



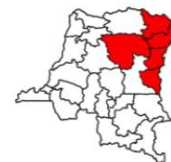
EBOLA PRESIDENTIAL FORCE TASK 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°092/MVEBDB/08/14/2026

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HIGHLIGHTS (last 24 hours)

Over the past 24 hours, 116 new confirmed cases and 58 deaths (38 community deaths and 20 intra-ETC) of Ebola Virus Disease (EVD) have been recorded in the provinces of Ituri (85 cases) and North Kivu (19 cases) and Haut Uélé (12). No new health zones have been affected.

ACCUMULATIVE CASE 4,843	ACCUMULATIVE CONFIRMED DEATHS 2,272	LETHALITY 46.9%	PATIENTS IN ISOLATION / CTE 777	ACCUMULATIVE HEALED 1006	TRACKING RATES CONTACTS (Of the Day) 83.2%
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Affected Provinces and Affected Health Zones



6 Provinces Affected

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo & Bas Uélé



55 Health Zones

Affected Out of 151

(36.4%)



Summary of key points from the last 24 hours

In the last 24 hours, 116 new confirmed cases and 58 deaths (38 Community deaths and 20 intra-ETC deaths from Ebola Virus Disease (EVD) have been reported in the provinces of **Ituri (85 cases), North Kivu (19 cases), and Haut-Uélé (12)**. Ituri accounts for approximately two-thirds of the new cases.

From the start of the epidemic until August 14, 2026, the Democratic Republic of Congo recorded 4,843 confirmed cases, including 2,272 deaths, representing a case fatality rate of 46.9%. Ituri remains the epicenter of the epidemic, accounting for 84.8% of cumulative confirmed cases and 73.3% of new cases reported in the last 24 hours.

At the end of the day on August 14, 2026, 30 patients were declared recovered after receiving two negative Ebola virus (EV) test results (27 in Ituri and 3 in North Kivu). However, 20 confirmed deaths within Ebola treatment centers were recorded, including 15 in Ituri and 3 in North Kivu. in Haut Uélé and 2 in North Kivu.

Overall, 55 of the 151 health zones in the six provinces have been affected since the start of the epidemic in the DRC. Ituri has 28 health zones affected out of 36, followed by North Kivu (12/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34) and Bas-Uélé (1/11).

Distribution of confirmed cases and deaths by affected province

(As of August 14, 2026)

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	4105	1,791	43.6%	28/36 (77.8%)	85
North Kivu	588	411	69.9%	12/34 (35.3%)	19
Haut Uélé	135	62	45.9%	6/13 (46.1%)	12
Tshopo	11	6	54.5%	7/23 (30.4%)	0
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Bas Uélé	1	1	100.0%	1/11 (9.1%)	0
Total	4,843	2,272	46.9%	55/151 (36.4%)	116

Confirmed cases and deaths by province and health zone (situation as of August 14, 2026)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	4105	1791	43.6%	85	31	15	46
Adja	11	1	9.1%				0
Aru	7	6	85.7%				0
Ariwara	7	2	28.6%				0
Aungba	11	5	45.5%	1	0		0
Bamboo	109	25	22.9%	5	3		3
Boga	1	1	100.0%				0
Bunia	1116	338	30.3%	16	5		5
Damascus	23	9	39.1%	2	0		0
Drodoro	13	6	46.2%				0
Fataki	63	30	47.6%	1	0		0
Gethy	4	1	25.0%	2	0		0
Kambala	2	0	0.0%				0
Kilo	31	12	38.7%				0
Komanda	45	31	68.9%	2	2		2
Lita	172	97	56.4%	3	0		0
Logo	11	5	45.5%				0
Lolwa	14	4	28.6%				0
Mahagi	4	1	25.0%				0
Mambasa	13	6	46.2%				0
Mandima	22	13	59.1%				0
Mangala	141	83	58.9%	19	7		7
Mongbwalu	584	282	48.3%	3	0		0
Nia-Nia	161	96	59.6%	5	4		4

Nizi	524	236	45.0%	11	5		5
Nyankunde	120	35	29.2%				0
Rimba	9	4	44.4%				0
Rwampara	838	324	38.7%	15	5		5
Tchomia	49	25	51.0%				0
To ventilate	0	113	N / A			15	15
North Kivu	588	411	69.9%	19	5	2	7
Blessed	90	66	73.3%	4	1		1
Butembo	110	91	82.7%	2	0		0
Goma	1	0	0.0%				0
Kalunguta	12	5	41.7%	1	0		0
Katwa	280	190	67.9%	6	1	1	2
Kyondo	15	12	80.0%	1	0		0
Lubero	1	1	100.0%				0
Mabalako	2	1	50.0%				0
Masereka	7	4	57.1%				0
Musienene	61	35	57.4%	5	3		3
Oicha	5	4	80.0%				0
Vuhovi	4	2	50.0%			1	1
Haut-Uélé	135	62	45.9%	12	2	3	5
Boma Mangbetu	22	8	36.4%			2	2
Gombari	1	1	100.0%				0
Isiro	39	19	48.7%				0
Pawa	19	14	73.7%				0
Rungu	1	0	0.0%				0
Wamba	53	20	37.7%	12	2	1	3
Tshopo	11	6	54.5%	0	0	0	0
Bafwasende	1	0	0.0%				0
Kabondo	3	2	66.7%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	3	3	100.0%				0
Mangobo	1	1	100.0%				0
Tshopo	1	0	0.0%				
Wanie-Rukula	1	0	0.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	1	1	100.0%	0	0	0	0
Buta	1	1	100.0%				0
Total	4843	2272	46.9%	116	38	20	58

*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

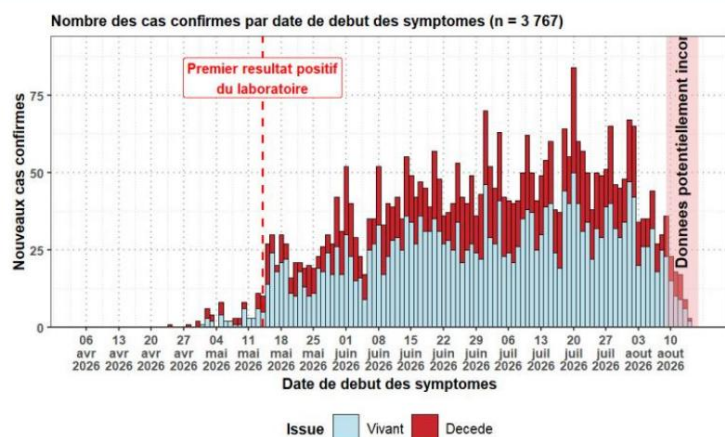
NA: Not Applicable

Status of alerts notified by province, as of August 14, 2026

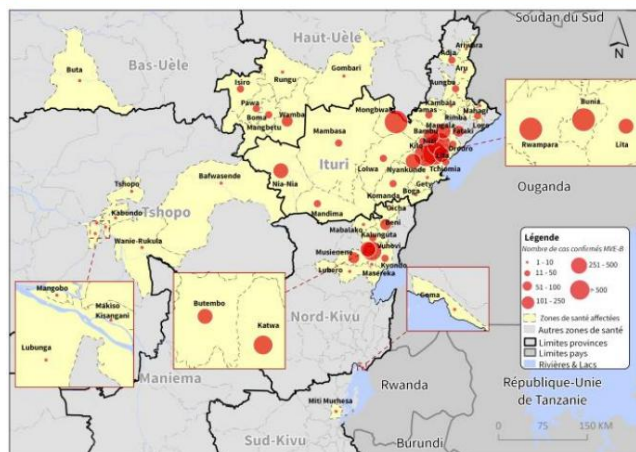
DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (non-suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Bas-Uele*	ND	ND	ND	ND	ND	ND	ND	ND	ND
Haut-Uélé	27	4	31	5	4	22	1	9	4
Ituri	557	85	642	147	79	217	6	157	138
North Kivu	937	53	990	114	53	812	53	167	103
South Kivu	36	0	36	1	0	35	0	1	1
Tshopo*	ND	ND	ND	ND	ND	ND	ND	ND	ND
Total General	1557	142	1,699	267	136	1086	60	334	246

*Alert data for Bas-Uélé and Tshopo are not available for the period considered.

Evolution of cases by date of symptom onset over the last 21 days



MAPPING OF CASES over the last 21 days



1. RESPONSE ACTIONS BY PILLAR

1.1. Coordination

1.1.1. Main Actions

- **North Kivu** : support in the cross-border surveillance mission in the Grand Virunga area and working session with SONAHU, CARITAS/Goma, FEPSI and WHH on their accreditations.
- **Ituri**: Bunia, under the chairmanship of the Military Governor of Ituri, General Kasongo, the province officially launched the Mass Administration of Antimalarial Drugs (AMMA) in the health zones of Bunia, Rwampara, Mongbwalu and Nyankunde:
Evaluation of the recommendations of the coordination meeting and inauguration of the new Ebola Treatment Centre (ETC) in Mongbwalu; reactivation of the silent health zones (Damas and Kambala) for the regular reporting of data.

1.2. Epidemiological surveillance

1.2.1. Main Actions

By the end of the day on August 13, 2026, 1,699 alerts had been recorded, of which 1,549 (91.2%) were verified in the four reporting provinces. Of these, 403 (26.0%) were confirmed as suspected cases and 334 (82.9%) were investigated, with 246 suspected cases transferred to COVID-19 treatment centers.

The proportion of contacts monitored in the last 24 hours is 83.2% (13,987 seen out of 16,802 to be monitored), close to the 85% threshold, with lower performance in Haut Uélé (61.6%) marked by insufficient internal completeness.

1.2.2. Challenges

Low reporting of alerts and listing and monitoring of contacts around confirmed cases.

1.3. Update on checkpoint and point of entry (PoC/PoE) activities

1.3.1. Main actions

- Seven (7) alerts were detected at the various PoE/PoC points: four in Ituri (three confirmed live alerts and one intercepted body), including two at the Mudzibala PoC (Nzere Health Zone, Bunia Health Zone) referred to the Elikya Emergency Response Center and one at the Poipo PoC (Abelkoko Health Zone, Mongbwalu Health Zone) referred to the Emergency Response Center. In North Kivu, three alerts were reported, including two at the Mavivi B PoC and one at the Kasitu PoC (Beni Health Zone), one of which was confirmed and two invalidated after investigation. No alerts were notified at the entry points of Haut-Uélé.
- In total, 221,313 travelers were screened in the two provinces that submitted daily PoE/PoC data (138,816 in Ituri and 82,497 in North Kivu), with a screening rate of 97.4%; South Kivu also reported 84,432 people screened, with a 100% screening rate. In Ituri, reporting completeness reached 100% (60/60 sites), with 97% of travelers screened, 97% having washed their hands, and 99% having received awareness training.
- In North Kivu, all 18 PoE/PoCs transmitted their data (100%): 80,462 out of 82,497 travellers were screened (97.6%), handwashing and awareness reaching the same level; a briefing of 15 providers from the Kasindi PoE and the Foner Kasindi PoC on surveillance, IPC and CREC was facilitated with IOM support.
- In Haut-Uélé, 10 PoE/PoC points are activated out of the 38 priority points (26.3%). Tshopo has not yet transmitted its PoE/PoC data for the period in question.

1.3.2. Challenges

- Insufficient operationalization of PoE/PoC: functionality remains low in Haut-Uélé, with only 26.3% of PoE/PoC activated, due to a lack of training, supervision and logistical and financial support; in Ituri, only one strategic site out of 11 is working without interruption and screening remains low at the Pont-Shari PoC due to a lack of involvement of uniformed personnel.
- Constraints affecting the functionality of PoE/PoC: 2,035 screening refusals (2.46%) were recorded in North Kivu, mainly in Matombo (668), Mukulya (363) and Makeke (289); in addition to this, there is non-payment of PoE/PoC providers in Ituri as well as in North Kivu, lack of mastery of surveillance tools by some providers and inconsistencies in alert data between PoE/PoC and health areas in the DHIS2 tracker.

1.4. Laboratory

1.4.1. Main actions

- Ituri recorded 93 positive results (336 samples analyzed), including 85 new cases (54 living and 31 deaths) and 8 new samples, while North Kivu confirmed 19 new cases including 5 deaths, and Haut-Uélé 12 confirmed cases and 5 deaths. Overall, 116 new cases

Cases of Ebola virus disease (EVD) have been confirmed in all three provinces; South Kivu recorded no positive results on the first sample received and analyzed.

1.5. Infection Prevention and Control (IPC/EDS)

1.5.1. Main actions

- **In Ituri**, 23 rings were actually opened out of 115 (39 reported the previous day and 76 cases retained by the pillar); 69 sites were identified and 69 decontaminated (100%), including 61 households and 8 care facilities, with no gap beyond 48 hours. The EDS teams recorded 72 daily death alerts and 23 postponements, for 76 EDS carried out (80%) and 69 bodies collected by swab (64 in households and 5 in care facilities).
- **In North Kivu**, the briefing of the EDS teams in the Beni health zone continued: 53 death alerts were received with 9 reports, 52 bodies were collected by swab, 50 EDS were carried out and 12 reports recorded; only one failure was reported, related to the refusal of the body bag in the Kazebere health area.
- **In Haut-Uélé**, 7 death alerts were recorded in 3 health zones (2 postponements and 5 alerts issued the same day) with 4 DHS assessments completed (57.1%) and 5 bodies collected by swab; 5 rings were opened, with decontamination of healthcare facilities in Pawa, Wamba (clinic and Ebola Treatment Center) and Isiro (university clinic), and an IPC assessment carried out at the Tely Health Center. Two bodies remained in the community due to a lack of samples.

1.5.2. Challenges

- Weak implementation of IPC standards: limited ring opening in Ituri (23 out of 115) due to the strike of decontamination workers (Nizi), insecurity (Komanda), refusal (Kilo) and addresses not found; completeness of IPC reporting limited to 9 out of 14 health zones in Ituri; low IPC scores of health facilities in Haut-Uélé (29% to 68%) with proven risk of exposure in six structures; insufficient means of transport dedicated to EDS activities in Haut-Uélé.

1.6. Continuity of care

1.6.1. Main actions

- **In Ituri**, 28 ambulances are available out of 32 allocated to the response: 43 trips were successfully completed (100%). Ebola treatment centers (ETCs) recorded 88 admissions, 27 recoveries, and 83 discharges in the last 24 hours; occupancy reached 545 patients out of 991 beds (55.0%), including 243 confirmed cases out of 497 beds (48.9%) and 297 suspected cases out of 494 beds (60.1%). The new ETC in Mongbwalu was inaugurated, and 250 units of blood were received from the National Blood Transfusion Center (CNTS); 502 patients (99%) received a hot meal.

In North **Kivu**, 56 new admissions were recorded (7 confirmed cases, including 6 at the Katwa Ebola Treatment Center and 1 at Vuhovi, and 49 suspected cases), with 42 discharges, including 37 non-cases, 3 recoveries, and 2 confirmed deaths (1 at Katwa and 1 at Vuhovi). Currently, 165 patients are hospitalized (31 confirmed and 134 suspected cases) out of 206 beds (80.1%), with 94.3% bed occupancy for suspected cases. Furthermore, 531 patients (100%) received a hot meal, and 63 healthcare providers were trained in the nutritional management protocol for Ebola.

- **In Haut-Uélé**, 13 new admissions were recorded (4 confirmed and 9 suspected) and no recoveries were discharged; 61 patients are hospitalized (37 confirmed and 24 suspected) in 84 beds (46 for confirmed cases and 38 for suspected cases), exceeding the capacity of the Wamba temporary isolation center. One new confirmed healthcare worker was admitted to the Isiro Ebola Treatment Center.
- **In South Kivu**, 6 suspected cases are being monitored in isolation at the Katana Health Zone (FOMULAC); no confirmed cases are being admitted to the CTE and the province has gone 65 days without a new confirmed case since June 10, 2026.

1.6.2. Challenges

Insufficient availability of referral resources (4 ambulances out of service in Ituri, only 5 ambulances in North Kivu and need for 10 more ambulances in Haut-Uélé) and low capacity of some CTE/CT: the CTEs in Bambu (confirmed and suspected) and Fataki are saturated, the occupancy of beds for suspected cases reaches 103% in North Kivu and the Wamba transitional isolation center is more than four times its capacity; absence of standardized CTEs in the six affected health zones of Haut-Uélé; high in-hospital mortality in Pawa (82.4%) and Isiro (51%).

1.7. Risk communication and community engagement

1.7.1. Main actions

- In Ituri, 898 new contacts were identified around the day's cases; 160 people were reached through home visits, 360 through educational talks, 110 through community dialogues, and 3,533 through mass awareness campaigns, which included 22 radio programs produced, 6 public service announcements broadcast, 36 community radio stations involved, and 42 IEC (Information, Education, and Communication) materials distributed. The malaria drug distribution campaign was launched in the health zones of Bunia, Rwampara, Nyankunde, and Mongbwalu.
- In North Kivu, 150 youth leaders and 50 motorcycle taxi parking managers in Beni were engaged in respecting prevention measures; communication around 27 confirmed cases reached 149 household managers, 14 rumors were clarified and 1 community incident resolved, while 14 resistances remain to be overcome in the Musienene health zone around the last five confirmed cases.
- In Haut-Uélé, 338 people were reached during home visits to 76 households, 862 during educational talks, and 59 during mass awareness campaigns; 5 radio stations broadcast prevention messages, and 41 IEC (Information, Education, and Communication) tools were distributed (total of 340). Ceremonies to mark the release of recovered patients were organized in the Wamba health zone.

1.7.2. Challenges

Persistent community challenges: refusal of decontamination motivated by the belief that "chlorinated water brings Ebola" in Ituri (Rwampara) and refusal of EDS in the Isiro health area in Haut-Uélé; 16 unresolved community incidents in North Kivu, including 14 in the Musienene health zone; low reporting of community feedback in Ituri (19% of health zones affected compared to 23% in the previous period); parallel uncoordinated interventions of awareness actors in Haut-Uélé and materials available only in French and Swahili.

1.8. Logistics

1.8.1. Main actions

In Ituri, the installed capacity reached 991 beds (497 for confirmed cases, 494 for suspected cases) with the inauguration of the new CTE in Mongbwalu; 23 out of 76 ring households (30%) received an IPC/WASH kit (8 handwashing devices in Bunia and 15 hygiene kits in Mongbwalu) and 250 bags of blood were received from the CNTS.

In North Kivu : delivery of infection prevention and control (IPC) supplies and epidemiological surveillance equipment to the health zones of Kalunguta, Mabalako, and Beni; delivery of mattresses and metal beds to the intensive care unit of Kyondo General Referral Hospital; deployment of nutritional support supplies to the Kibua health zone via IMA; and continued construction of the triage area at Vuhovi General Referral Hospital by ULB Cooperation. In Haut-Uélé: packaging and delivery of care, IPC, and EDS equipment to the health zones of Pawa and Isiro; and acquisition of 600 liters of fuel with support from MEMISA.

1.8.2. Challenges

Ongoing logistical needs to support the expansion of the response, including the completion of the extensions to the Nizi, ISTM and Bambu CTEs, and the continued supply

essential inputs and PPE — with shortages reported at the North Kivu provincial depot — the availability of means of transport and evacuation, and a fuel stock alert in Haut-Uélé.

1.9. Security 1.9.1.

Main actions

- Securing the response activities continued in North Kivu with three community policing initiatives that were fully implemented: security coverage for decontamination teams at the Sagesse Divine dispensary and the Naissance Désirable health center (AS Butsili), and support for the EDS team in Kididiwe. EDS teams were also accompanied for the burials of confirmed cases in Kamiriki/Kyondo, Kyambogho, and Cugeki; 147 soldiers received awareness training at the Mambango parade, and 121 women participated in an educational session.
- Persistent insecurity and community resistance during dignified burials and secured, limiting access for response teams; insecurity reported in some villages opening rings in Ituri (Komanda); seventeen health areas identified as problem areas in North Kivu; lack of support for police officers assigned to the CTEs in Katwa and Kitumba.

1.10. PSEA (Prevention of Sexual Exploitation and Abuse)

1.10.1. Main actions

- Continued awareness-raising activities on Prevention of Sexual Exploitation and Abuse (PSEA): In Ituri, a two-day training session brought together 50 participants (21 women and 29 men) in the Tchomia health zone, with all participants signing the code of conduct and 13 banners being distributed; an awareness session on PSEA and the toll-free hotline reached 20 people in Mongbwalu. In total, 471 people were reached in the community during the week, and two partners reported on their PSEA activities.

1.10.2. The challenges

Coverage of awareness-raising activities and strengthening of mechanisms for the prevention, reporting and management of cases of sexual exploitation and abuse is still insufficient in all areas concerned by the response.

KEY MESSAGES

- Ebola virus disease transmission remains active, with 116 new cases and 58 deaths, including 38 fatalities community and 20 intra-CTE recorded in the last 24 hours, bringing the national total to 4,843 confirmed cases and 2,272 deaths.
- Ituri remains the epicenter of the epidemic, concentrating 84.8% of cumulative confirmed cases and 73.3% of new cases reported in the last 24 hours and a greater expansion (28 of the 36 health zones affected).
- Surveillance shows 94.5% of suspected cases investigated while the follow-up rate (83.2%) remained below the threshold of 85.0%.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of coordination and support mechanisms.



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